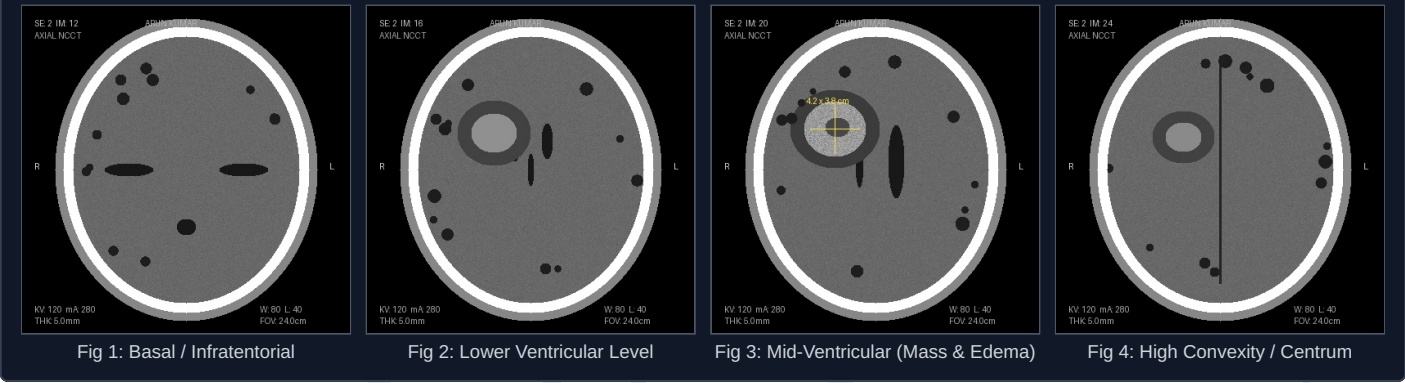




COMPUTED TOMOGRAPHY (CT) EXAMINATION REPORT — BRAIN / HEAD

Patient Name:	Arun Kumar	Patient ID / UHID:	CT-2026-08421
Age / Gender:	52 Yrs / Male	Study Date & Time:	29-Aug-2026 • 11:24 AM
Referring Physician:	Dr. R. Sharma	Modality / Protocol:	64-Slice CT / NCCT Head
Examination:	CT Brain (Plain / Non-Contrast)	Report Status:	Preliminary / Sample Report

REPRESENTATIVE KEY AXIAL CT IMAGES (STANDARD BRAIN WINDOW W:80 L:40)



CLINICAL INDICATION & HISTORY

52-year-old male presenting with progressive, worsening headaches of several weeks duration, associated with intermittent morning nausea, episodes of mild lightheadedness, and recent onset of cognitive slowing/difficulty concentrating. Evaluated for suspected intracranial pathology.

TECHNIQUE & PROTOCOL

Contiguous volumetric non-contrast axial computed tomography (NCCT) sections of the brain were obtained from the skull base to the vertex using a 64-slice helical CT scanner (120 kVp, auto-mA modulation, 5.0 mm slice reconstruction, matrix 512 × 512). Multiplanar coronal and sagittal reformations were generated and reviewed in dedicated brain, soft tissue, and bone window algorithms. No intravenous iodinated contrast was administered during this study.

RADIOLOGICAL FINDINGS

- Intracranial Mass Lesion:** There is an approximately **4.2 × 3.8 cm** well-defined, heterogeneous mass-like lesion centered within the **right frontal cerebral hemisphere**.
- Perilesional Edema:** The lesion is surrounded by a prominent halo of hypodense vasogenic edema involving the subcortical and deep white matter of the right frontal lobe.
- Mass Effect & Ventricles:** Mild local mass effect is demonstrated, evidenced by partial effacement of the adjacent right frontal cortical sulci and slight compression/contour deformity of the frontal horn of the right lateral ventricle. The left lateral ventricle, third ventricle, and fourth ventricle maintain normal morphology and baseline caliber. No significant transfalcine herniation or midline shift is evident (midline alignment preserved, shift < 2 mm).
- Hemorrhage & Caliber:** No acute hyperdense intracranial hemorrhage, gross macro-calcification, or acute extra-axial fluid collections (epidural/subdural) are identified.
- Infratentorial Structures:** Cerebellar hemispheres, vermis, and brainstem demonstrate expected normal CT attenuation and morphology. The basal cisterns and cerebellopontine angles are patent bilaterally.
- Calvarium & Extracranial:** Visualized bony calvarium, skull base, and overlying scalp soft tissues are intact with no lytic or sclerotic osseous lesions. Visualized paranasal sinuses and mastoid air cells are well-aerated.

IMPRESSION

- Right frontal intracranial mass lesion (measuring ~4.2 × 3.8 cm)** associated with surrounding hypodense vasogenic edema and mild local mass effect on the right lateral ventricle.
- The imaging appearance is **concerning for a primary intra-axial intracranial neoplasm** (e.g., high-grade glial series neoplasm) or solitary secondary lesion.
- No acute intracranial hemorrhage or significant midline shift demonstrated on this non-contrast examination.

RECOMMENDATION & NEXT STEPS: Non-contrast CT is not definitive for precise histopathological characterization. Urgent **Contrast-Enhanced Magnetic Resonance Imaging (CEMRI Brain with MR Spectroscopy/Perfusion)** is strongly recommended for detailed multiplanar structural assessment, along with immediate specialist **Neurology / Neurosurgical evaluation**.

Dr. R. Sharma

Dr. R. Sharma, MD, DNB

Consultant Radiologist & Neuroimaging Specialist

Reg. No: MCI-2014-98452 • Dept. of Radiodiagnosis

Exam Date: 29-Aug-2026 11:24 IST • **Verified Date:** 29-Aug-2026 12:15 IST
Electronically verified & signed via PACS Enterprise • Authentication ID: RAD-AUTH-88421

NOTICE: THIS IS A SYNTHETIC / SIMULATED RADIOLOGY REPORT DESIGNED FOR EDUCATIONAL, CLINICAL DEMONSTRATION, OR SOFTWARE TESTING PURPOSES ONLY. NOT A VALID MEDICAL RECORD.

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MEDICAL REPORT